

MENTAL HEALTH & SHOCK STATES

Hypovolaemic Shock | Acute Stress | PTSD | Psychosis | Suicide Risk | Grief

Clinical Guideline for AI Model Training Data — Version 1

Sources: WHO mhGAP | MSF Mental Health Guidelines | IASC MHPSS | Sphere Handbook | Hesperian Health Guides

In conflict and disaster settings, both physiological shock and psychological trauma are common and often occur together. This guideline covers recognition and low-resource management of hypovolaemic shock, and the most common acute and chronic mental health presentations in affected populations. Mental health support does not require specialist training — empathic listening, safety, and practical support are the foundation.

1. HYPOVOLAEMIC SHOCK — RECOGNITION & MANAGEMENT

Hypovolaemic shock results from loss of blood or fluid volume. In conflict settings, haemorrhage is the most common cause. Early recognition and haemorrhage control are life-saving.

1.1 Recognition — Shock Classification

Class	Blood Loss	Pulse Rate	BP	Consciousness	Field Signs
Class I (compensated)	< 750 mL (< 15%)	Normal (< 100)	Normal	Normal / slightly anxious	Minimal signs — may appear normal
Class II	750–1500 mL (15–30%)	100–120	Normal or slight drop	Anxious, restless	Pale, cold clammy skin; thirsty
Class III	1500–2000 mL (30–40%)	120–140	Reduced	Confused	Very pale; rapid weak pulse; reduced urine output
Class IV (severe)	> 2000 mL (> 40%)	> 140 or undetectable	Very low or absent	Lethargic/unconscio us	Extreme pallor, gasping, imminent death

1.2 Haemorrhage Control — Priority Actions

STEP 1	DIRECT PRESSURE Apply firm direct pressure over the wound with the cleanest available cloth. Maintain continuous pressure for minimum 10 minutes — do NOT lift to check.
STEP 2	TOURNIQUET (LIMB HAEMORRHAGE) Apply tourniquet 5–7 cm above wound for life-threatening limb haemorrhage. Note exact time of application. Commercial tourniquet or improvised from belt or cloth twisted with a stick.
STEP 3	WOUND PACKING (JUNCTIONAL WOUNDS) For wounds that cannot be tourniqueted (groin, armpit, neck): pack wound firmly with the cleanest available cloth; maintain firm pressure.
STEP 4	POSITION Lay patient flat; elevate legs 15–30 cm if no spinal or chest injury — helps maintain perfusion to brain and vital organs.
STEP 5	KEEP WARM Cover with blankets — hypothermia worsens coagulopathy and accelerates death in haemorrhagic shock.

STEP 6	ORAL FLUIDS (CLASS I/II ONLY)
	If conscious and no abdominal injury: small sips of clean water with a pinch of salt and sugar. Do NOT give oral fluids if Class III/IV or if surgery likely.

STEP 7	URGENT REFERRAL
	Severe haemorrhagic shock requires surgical haemorrhage control. Transport immediately.

DO NOT remove a tourniquet in the field — hospital assessment required before removal.

DO NOT give IV fluids excessively before surgical haemorrhage control — may dislodge clots.

DO NOT administer oral fluids to a patient with suspected abdominal injury — masks signs and complicates surgery.

2. ACUTE STRESS REACTION

Acute stress reaction (ASR) is a normal psychological response to an abnormal event. It is expected after combat, bereavement, or witnessing violence. Most people recover naturally within days.

Feature	Description
Emotional symptoms	Anxiety, fear, panic, anger, tearfulness, emotional numbness, feeling detached from surroundings
Physical symptoms	Palpitations, trembling, sweating, nausea, headache, dizziness
Cognitive symptoms	Difficulty concentrating, confusion, disorientation, intrusive memories
Behavioural symptoms	Withdrawal, hyperactivity, risk-taking, searching behaviour (in bereavement)
Duration	Hours to days — if > 4 weeks, consider PTSD

2.1 Psychological First Aid (PFA) — Low-Resource Approach

STEP L	LOOK
	Observe the person. Are they safe? Are they injured? Are they in acute distress? Do they need immediate practical help?
STEP L	LISTEN
	Approach calmly. Introduce yourself. Sit at eye level. Do not rush. Ask open questions: 'How are you feeling?' Do not force the person to talk.
STEP L	LINK
	Help them access practical support: food, water, shelter, family contact, medical care. Link to community and family.

- Do NOT debrief — do not ask the person to retell traumatic events in detail. This can worsen distress.
- Do NOT give false reassurance ('everything will be okay').
- Do validate feelings: 'It is completely normal to feel this way after what you have been through.'
- Ensure basic needs first: safety, water, food, warmth, contact with family.
- Most people recover without formal mental health treatment.

3. POST-TRAUMATIC STRESS DISORDER (PTSD)

PTSD may develop weeks or months after traumatic events. It is distinct from normal grief or sadness.

Symptom Cluster	Common Presentations
Re-experiencing	Flashbacks (involuntary vivid memories of trauma); nightmares; emotional distress triggered by reminders

Avoidance	Avoiding thoughts, feelings, people, or places associated with trauma
Hyperarousal	Constant alertness (hypervigilance), easily startled, difficulty sleeping, irritability, anger outbursts
Negative cognition	Persistent guilt, shame, self-blame; negative beliefs about self or world; emotional numbness
Diagnosis requires	Symptoms > 1 month; significant impairment in daily functioning

3.1 Low-Resource Support

- Psychoeducation: explain that PTSD is a normal response to abnormal events — not a sign of weakness.
- Re-establish routine: regular daily activities, meals, sleep schedule.
- Social support: encourage connection with family, community, and cultural or religious practices.
- Physical activity: regular walking or exercise significantly reduces PTSD symptoms.
- Relaxation techniques: slow deep breathing (4 counts in, hold 4, out 4); grounding techniques (name 5 things you can see, 4 you can touch, 3 you can hear).
- Refer for psychological therapy (CBT, EMDR) if available — these are first-line treatments.
- mhGAP guidelines: frontline workers can deliver brief structured interventions without specialist training.

4. ACUTE PSYCHOSIS

Psychosis involves loss of contact with reality. It may be triggered by extreme stress, sleep deprivation, substance use, or underlying psychiatric illness. In conflict settings it may be mistaken for possession or spiritual illness.

Feature	Description
Hallucinations	Hearing voices or seeing things that others cannot perceive
Delusions	Fixed false beliefs — may believe they are being followed, have special powers, or that others are trying to harm them
Disorganised speech	Incoherent, jumping topics; words not forming meaningful sentences
Disorganised behaviour	Unpredictable or bizarre behaviour; difficulty with self-care
Negative symptoms	Flat affect, withdrawal, lack of motivation, reduced speech

4.1 Management

- Ensure safety of the person and others.
- Calm, quiet environment — reduce stimulation.
- Speak slowly and clearly in short sentences; do not argue with delusions.
- Maintain a non-threatening physical distance.
- Do not leave acutely psychotic person alone.
- Antipsychotic medication is required for treatment — refer to mental health services.
- Acute psychosis may have medical causes: head injury, high fever, meningitis, substance use — screen for these.

5. SUICIDE RISK ASSESSMENT & RESPONSE

Suicide risk is elevated in conflict settings, particularly among those who have lost family, been displaced, or experienced sexual violence. Asking about suicide does NOT increase the risk.

5.1 Risk Assessment

Risk Level	Indicators	Response
Passive ideation	Wishes to be dead; feels life is not worth living; passive statements ('I wish I could disappear')	Listen; do not leave alone; assess for plan
Active ideation without plan	Thoughts of killing self but no specific plan or intent	Immediate support; remove means; refer to mental health

Active ideation with plan	Specific plan, intent, access to means	Do NOT leave alone; immediate referral; remove means
Attempt or imminent risk	Has made attempt or actively engaging in suicidal behaviour	Medical emergency + psychiatric emergency; constant supervision; urgent referral

5.2 Safe Communication

- Ask directly and calmly: 'Are you having thoughts of ending your life?' — direct questions do not increase risk.
- Listen without judgment; acknowledge the pain.
- Do NOT leave a high-risk person alone.
- Remove access to means: weapons, medications, sharp objects, ropes.
- Involve family or trusted community members if the person consents.
- Make a safety plan: identify warning signs, coping strategies, and who to contact in crisis.
- Refer to mental health specialist or community support worker.

NEVER promise confidentiality about a suicide plan — safety must come first.

Removing the means (weapons, medications) is one of the most effective single interventions.

Do NOT dismiss suicidal statements as 'attention-seeking'.

6. GRIEF & BEREAVEMENT IN CONFLICT

- Grief is a normal response to loss of family, home, or community.
- Normal grief: waves of sadness; crying; preoccupation with the deceased; may last months to years.
- Complicated grief: persistent, disabling grief lasting > 6 months with significant impairment.
- Cultural and religious mourning practices are important — support and facilitate where safe.
- Allow the person to talk about the deceased if they wish — do not redirect away from grief.
- Do not say 'I know how you feel' — acknowledge that grief is unique.
- Refer if: persistent inability to function; suicidal ideation; prolonged inability to eat or care for self.

7. RED FLAGS — CRITERIA FOR URGENT REFERRAL

Red Flag Criterion	Clinical Significance / Action
Class III/IV haemorrhagic shock	Surgical haemorrhage control required immediately
Active suicidal plan or attempt	Psychiatric emergency — constant supervision; immediate referral
Acute psychosis with risk to self/others	Psychiatric emergency — safe environment; urgent referral
Severe PTSD with functional collapse	Cannot care for self or children — urgent mental health referral
Acute stress reaction with physical symptoms	Exclude medical causes (hypoglycaemia, cardiac, toxic)
Disorganised psychosis with fever/head injury	May have organic cause — urgent medical assessment

REF. REFERENCES

- [1] WHO. mhGAP Intervention Guide for Mental, Neurological and Substance Use Disorders. Version 2. Geneva: WHO, 2016.
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- [3] MSF. Mental Health and Psychosocial Support in Humanitarian Settings. MSF, 2022.
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- [5] WHO. Psychological First Aid: Guide for Field Workers. Geneva: WHO, 2011.
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- [7] PAHO/WHO. Mental Health in Emergencies. 2020.

This document was compiled strictly from the sources listed above for use as AI model training data in clinical emergency assessment. No clinical decisions should be made based on this document alone without consulting qualified medical professionals.